



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ماجده محمد حسن عبد النبي

المحافظة : الجيزة الحي / المدينة : الحوامديه العنوان : -

رقم البطاقة : 27309142100285 رقم التليفون : 01112710679 تاريخ تسجيل الإستمارة : 04-10-2026

العمر (تاريخ الميلاد) : 14-09-1973 المهنة : بدون عمل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: ماجده محمد حسن عبد النبي National ID: 27309142100285	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Dms.
 = Logo
 عربي
 كويتي



Patient Name: Magda Mohamed Hassan A. Naby	Date : 13/11/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 52 Year(s)
Medical ID: 8969	Pat. ID: 132912

**Bilateral Digital soft tissue mammography with high resolution
breast sonography revealed:**

Clinical summary: 52-year-old female with negative family history, complaining of left breast lump of four months duration.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast parenchyma with mottled appearance yet no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Scattered fibroglandular densities. (ACR b)
- outer central, posterior third, irregular high density mass lesion with partially obscured margins , surrounding distortion and inseparable anterior focal asymmetry seen involving the LOQ and to a lesser extent the LIQ. No suspicious microcalcifications.
- Related overlying thickened skin and coarsened trabeculae are noted
- **Ultrasound** demonstrated 3-4 o'clock, posterior third, 6 cm from nipple, 5 mm deep to the skin, irregular spiculated heterogenous hypoechoic solid mass lesion with surrounding echogenic halo, posterior acoustic shadowing and minimal internal vascularity on color Doppler study measuring 33 x 35 mm. anterior related prominent pathological ducts are noted reaching 2 mm in caliber some showing soft content within could be traced till the nipple.
- Thickened overlying skin reaching 8 mm with minimal interstitial edema and coarsened trabeculae involving the lower quadrants.
- Multiple (more than nine) discrete pathological LNs are noted , involving axillary levels 1, 2 & 3 as well as supraclavicular ;the largest measuring 19 x 20 mm.

Conclusion:-

Right breast (BIRADS 1): Normal study.

Left breast (BIRADS 5): LOQ suspicious mass lesion with surrounding focal edema pattern & ipsilateral multiple pathological axillary and supraclavicular lymph nodes .

Recommendation:-

For biopsy and histopathological correlation.

Best Regards,
DR. Amal Hegazi, M.Sc.

Name:	ماجده محمد حسن عبدالنبي	
MR. :	8202	
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient has a history of hypertension and prior spinal surgery with left breast cancer staged as cT4 N3 M0. She underwent left L3 to S1 screw fixation in April 2026. On examination there is left breast edema, left breast induration, and palpable left axillary lymph node measuring 2 × 2 cm as well as a left supraclavicular lymph node 2 × 2 cm. Multidisciplinary discussion recommended neoadjuvant therapy with trastuzumab, pertuzumab, and docetaxel (AC + Herceptin + Perjeta). The patient's weight is 125 kg, height 155 cm, and performance status ECOG 1. The planned regimen includes Adriamycin 120 mg/m² and cyclophosphamide (Endoxan) 1200 mg/m² for cycle 1. The patient has completed cycle 1 of this regimen. The patient presented with grade 2 nausea and vomiting despite supportive anti-emetics. If intolerance persists, a switch to taxanes (Phesgo) will be considered. The current regimen for cycle 2 consists of Adriamycin 105 mg/m² and Endoxan 1050 mg/m². The patient has completed cycle 2 of this regimen. On examination there is persistent left breast edema and induration. The patient reports ongoing perianal pain; she stopped trastuzumab for one month due to a perianal abscess that was surgically evacuated and has now improved. For cycle 3 the patient received Adriamycin 105 mg/m² and Endoxan 1050 mg/m² and will receive Phesgo and Taxol in the next cycle. The patient has completed cycle 3 of this regimen.		
تحريراً في ,		
	الوقت :	التاريخ : 19-09-2026
	dina makram	الطبيب المعالج :
يعتمد ,		

Patient Name : الأستاذ / ماجده محمد حسن :	Request No. : 13003618
Patient No. : 27309142100285	Request Date : 25/11/2025
Path No. : 3662/25	Verification Date : 02/12/2025
Sex : Female	Financial Category : جوستاف روسي - مبادره
Age : 52 Y	Department : Out Patient
Referreing Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, Collected on 25/11/2025, at 13:20 pm, Cold ischemia time: 30 seconds.

Gross Examination:

- Multiple fragmented grey white tissue cores, the longest is 1 cm. Totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for:

- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:

* Histological type: Invasive duct carcinoma.

* Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).

* DCIS: Absent.

* Angioinvasion: Absent.

* Perineural invasion: Absent.

* IPT (Positive control showed appropriate reactivity):

- ER: Strong nuclear reaction in 15% of tumor cells (score 6/8).

- PR: Negative nuclear reaction in tumor cells (score 0/8).

- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).

- Ki67%: 30%.

Diagnosis:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 2.

IPT:

Verified by :

Dr.Duaa Mohamed



Patient Name : الأستاذة / ماجده محمد حسن	Request No. : 13003618
Patient No. : 27309142100285	Request Date : 25/11/2025
Path No. : 3662/25	Verification Date : 02/12/2025
Sex : Female	Financial Category : جوستاف روسي - مبادره
Age : 52 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- ER: Positive (score 6/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67%: 30%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 14/12/2025 12:08:12

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذه / ماجده محمد حسن	Request No.	: 13003618
Patient No.	: 27309142100285	Request Date	: 25/11/2025
Path No.	: 3662/25	Verification Date	: 02/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 52 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Verified by :
Dr.Duaa Mohamed

Printing Date : 14/12/2025 12:08:12

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Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name : MAGEDA MOHAMED
HUSSIEN

Date : 27-12-2025 03:48

Branch : Portable

Medical No. : GRI-132912

Ref.Physician Prof :

Ins.Company :

Left axillary lymph nodes.

Dr. Mai Seddik

**MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER
BOTH ORAL & I.V. CONTRAST ADMINISTRATION: -**

Findings:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- Bone window: L3 to S1 transpedicular screw fixation.

Opinion:

- **Normal CT scan of the abdomen and pelvis.**
- Bone window: L3 to S1 transpedicular screw fixation.

Dr Mai Mohamed MD

Name : MAGEDA MOHAMED

Date : 27-12-2025 03:48

HUSSEIN

Branch : Portable

Medical No. : GRI-132912

Ref.Physician Prof :

Ins.Company :

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH
CORONAL RECONSTRUCTION: -**

Findings:

- Left breast outer quadrant soft tissue lesion measures 2.3x3 cm.
- Left axillary lymph nodes 19x23 mm.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Upper abdominal cuts are unremarkable.

OPINION:

- Left breast outer quadrant soft tissue lesion for US correlation.

ECHO REPORT

NAME: محمد عيسى

DATE: 7/10/2026

ID NUMBER: /

SEX:

AGE:

F	62%	55-75%	PWT	0.8	0.6-1.1cm
VEDD	4.7	3.5-5.8cm	AO.R	2.3	3-3.7cm
VESD	2.3	2-4cm	LA	3.8	2-4cm
S	28%	25-45%	RV		0.7-2.3com(supine)
WT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function GI

SWMA No RWMA

LA and Ao Root Dimensions Normal

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

TAPSG: 2.5

DR. محمد عيسى